



GUIDELINE

Agitation and Arousal Management for Mental Health Inpatients

Scope (Staff):	CAMHS clinical staff
Scope (Area):	CAMHS Inpatient Service (Ward 5A and Nickoll Ward) or other hospital location with dedicated mental health in-reach services

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Aim

This guideline outlines the stepwise management of acute agitation and arousal in children admitted to the CAMHS Inpatient Service or other hospital location with dedicated mental health in-reach services.

Risk

Non-adherence to this protocol could result in harm to patients or staff due to further escalation of arousal or clinical deterioration.

Definitions

CAMHS Inpatient Service: refers to both Ward 5A and Nickoll Ward at Hollywood Private Hospital.

Child: an infant, child, adolescent or young person under the age of 18 years.

Therapeutic Crisis Intervention (TCI): TCI is a crisis prevention and intervention model developed by Cornell University for residential child care facilities. It's application has been generalised to the clinical setting as well. The TCI protocol assists in:

- Preventing crises from occurring
- De-escalating potential crises.
- Effectively managing acute crisis phases.
- Reducing potential and actual injury to children and staff.
- Learning constructive ways to handle stressful situations.
- Developing a learning circle within the organization.

Key points

When managing an acutely aroused and agitated patient, medication must be used:

- as an adjunct to other interventions (e.g. behavioural interventions, TCI); and
- in accordance with the CAHS [Agitation and Arousal Medication Management Guideline](#).

Process

The following process outlines the steps that must be taken when a child is escalating in emotion and behaviour towards the point of a crisis. Staff must use de-escalation strategies prior to moving through each of these steps.

The following process is summarised in [Appendix 1: De-escalation flowchart](#).

Step 1: De-escalation

a) Where possible, identify the cause of agitation

Causes of or contributors to acute agitation for children may include:

- Acute anxiety (anxious, phobic, or unwilling patient);
- Patient presenting with externalising/challenging behaviour;
- Acute alcohol and/or drug intoxication;
- Post-Traumatic Stress Disorder (PTSD) following acute trauma (psychological, physical, sexual);
- Psychotic disorders (schizophrenia, severe depressive disorder, mania, drug-induced psychosis, organic disorders);
 - Delirium; and
 - Other organic brain syndromes (e.g. post head injury, tuberous sclerosis, frontal lobe tumours, etc.)

b) Adopt de-escalation techniques/strategies

Techniques and strategies may include:

- Self-awareness and environmental strategies
- Am I calm enough to deal with this situation and to build an emotional bridge with the young person?
- What does this child feel, need, expect and want?
 - Look beyond the behaviour in front of you; use your active listening skills to seek the child's perception of the situation and reason for distress
- How is the environment affecting the situation?
 - Ensure a safe physical environment - are there dangerous items that need removal?
 - Is the environment overstimulating for the child (e.g. due to noise or overcrowding)? – if so, relocate to a quieter environment with low stimuli, if possible.
- Non-verbal strategies (what do I do?)
 - Take a deep breath;
 - Step back (keep a safe distance and know your points of egress);
 - Give the situation time (de-escalation can take time);
 - Sit down (only if it is safe to do so); and
 - Adopt open and non-threatening body language, with facial expressions that convey genuine understanding.
- Verbal strategies (what do I say)?
 - Say very little; your facial expressions, manner and tone of voice convey more meaning than words. However, some factual information may be helpful to the young person in distress (e.g. "we are currently contacting your parents"; "we are making arrangements about family visits").
 - Consider your facial expressions.
 - Use an even tone of voice.
 - Maintain a calm manner.
 - Using clear, simple language, provide empathetic responses that:
 - Validate feelings: E.g. "I can see how difficult it is for you right now"
 - Encourage positive behaviours: E.g. "When you are ready we can talk"
 - Emphasise desirable outcomes: E.g. "I know we can work through this together"
 - Offer an apology: E.g. "I am sorry that you feel you have been ignored"

Step 2: Use of medication for acute agitation and arousal

Staff must refer to the CAHS [Agitation and Arousal Medication Management Guideline](#) for advice regarding the type, dose and monitoring requirements of medications used for rapid tranquilisation.

a) Arousal rating – Sedation Assessment Tool

- Nursing staff must record pre and post patient arousal rating scales on the Agitation and Arousal Medication Chart. If the chart is not available, the ratings must be recorded in the child's progress notes.
- For per oral (PO) medications, responsiveness must be measured and recorded at baseline and 30-60 minutes after administration, and repeated as necessary.
- For intramuscular injections (IMI), responsiveness must be measured and recorded at baseline and 30 minutes after administration, and repeated as necessary.
- Staff must use the following table to measure and record responsiveness:

Sedation Assessment Tool		
SCORE	RESPONSIVENESS	SPEECH
+3	Combative, violent, out of control	Continual loud outbursts
+2	Very anxious and agitated	Loud outbursts
+1	Anxious and restless	Normal/Talkative
0	Responds easily to name	Speaks Normally
-1	Asleep but rouses if name is called	Slurring or prominent slowing
-2	Physical stimulation	Few recognisable words
-3	No response to stimulation	Nil

Compliance monitoring

Where failure to comply with this procedure or application of the strategies outlined in this procedure have been unsuccessful and resulted in an incident of aggression or assault, the incident will be reported through Datix CIMS and monitored by CAMHS Executive Committee.

Any incident that meets the criteria for a notifiable incident as defined by the [Mental Health Act 2014](#) (WA), must be reported to the Chief Psychiatrist in accordance with the [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#). The Office of the Chief Psychiatrist actively monitors and reviews reported notifiable incidents for all Health Service Providers.

Related CAHS internal policies, procedures and guidelines
<u>CAHS Allergy and Adverse Drug Reaction Management Policy</u>
<u>CAHS Agitation and Arousal Medication Management Guideline</u>
<u>CAHS High Risk Medicines Policy</u>
<u>CAHS Recognising and Responding to Acute Deterioration Policy</u>
<u>CAHS Restraint of Patients/Clients Policy</u>
<u>CAMHS Consent Guideline</u>
<u>CAMHS Physical Health Assessment and Monitoring Policy</u>
<u>CAMHS Physical Restraint Policy</u>
<u>CAMHS Psychotropic Medication - physical health monitoring handbook</u>
<u>CAMHS Psychotropic Medication – monitoring adverse physical health effects</u>
<u>CAMHS Seclusion Policy</u>
<u>CAMHS Risk Assessment and Management Policy</u>
<u>PCH Allergy and Adverse Drug Reaction Documentation and Reporting Procedure</u>

Related external legislation, policies, and guidelines
<u>Medicines and Poisons Act 2014</u>
<u>Mental Health Act 2014 (WA)</u>
<u>Clinical Incident Management Policy</u> - MP 0122/19
<u>Consent to Treatment Policy</u> - MP 0175/22
<u>High Risk Medication Policy</u> - MP 0131/20
<u>Medication Chart Policy</u> - MP 0078/18
<u>Medication Review Policy</u> - MP 0104/19
<u>Safety Planning for Mental Health Consumers</u> – MP 018/24
<u>Statewide Medicines Formulary Policy</u> - MP 0077/18
<u>Recognising and Responding to Acute Deterioration (RRAD) Policy</u> - MP 0171/22

References
Adiamando et al. Paediatric Emergency Care Volume 26, Number 11, November 2012.
Therapeutic Guidelines – Antipsychotic Adverse Effects
Bridging the Gap from Evidence to Policy to Practice – reducing the progression to metabolic syndrome for children and adolescents on antipsychotic medication
The Royal Children’s Hospital Melbourne: Acute behavioural disturbance: Acute management
Guidelines for the Management of Acutely Disturbed Children and Adolescents (aged 6-17 years), East London NHS Trust.
South West London and St George’s Mental Health Trust NHS
International Diabetes Federation - Metabolic syndrome in children and adolescents.
Calver, LA, Stokes, B, Isbister, GK Sedation assessment tool to score acute behavioural disturbance in the emergency department, Emerg Med Australas. 2011 Dec;23(6):732-40
NICE Clinical Guideline 25: The short term management of disturbed/violent behaviour in inpatient psychiatric settings and emergency departments. February 2005
Nobay, F, Simon BC, Levitt, A et al. A prospective double-blind, randomised trial of Midazolam versus Haloperidol versus Lorazepam in the chemical restraint of violent and severely ill patients. Academy of Emergency Medicine, 2004, 11, 744-749.
Paediatric Formulary Committee. BNF for Children 2008. London: BMJ Publishing Group, RPS Publishing, and RCPCH Publications; 2008.
Policy for the use of Rapid Tranquilisation for Acutely Disturbed Young People Aged 6-17 years: Barnet, Enfield & Haringey, NHS.
Rapid Tranquilisation Article from Patient.co.uk accessed 22 October 2015 online http://patient.info/doctor/Rapid-Tranquilisation.htm
Rapid Tranquilisation Policy for Use in Children and Young People Aged 12 to 18 Years, NHS Plymouth, Version No 1 (August 7 2009)
Rapid Tranquilisation Policy for Young People aged 6-17 years: Suffolk Mental Health Partnership Trust NHS.
Strategies for the management of disturbed and violent patients in psychiatric units. Council Report CR 41 March 1995
Therapeutic Crisis Intervention (TCI) Overview ". Cornell University. Archived from the original on 5 August 2004.
WA Therapeutics Advisory Group. Guidelines .
Women and Children’s Health Network - Antipsychotic physical health and adverse effect monitoring package.

Useful resources (including related forms) (if required)	
CAHS PCH MR816.06 Agitation and Arousal Medication Chart	
CAMHS MR505.01 Antipsychotic Physical Health Monitoring Form	

This document can be made available in alternative formats on request.

File Path:	CD/25/90160		
Document Owner:	Director Clinical Services		
Reviewer / Team:	CAMHS Policy, CAMHS Safety and Quality, A/Director CAMHS Acute Inpatient, Head of Department CAMHS Acute, Head of Service CAMHS Acute Inpatient, CAHS Medication Safety Pharmacist, CAMHS Medication Safety Committee.		
Date First Issued:	July 2016	Last Reviewed:	02/10/2025
Amendment Dates:	May 2019, November 2024	Next Review Date:	02/10/2028
Approved by:	Director Clinical Services	Date:	01/09/2025
Endorsed by:	CAMHS Clinical Governance Executive Committee CAMHS Medication Safety Committee	Date:	10/09/2025 02/10/2025
Aboriginal Impact Statement and Declaration (ISD)		Date ISD approved:	04/09/2025
Standards Applicable:	NSQHS Standards:  Child Safe Principles: 1, 8, 10		
Printed or personally saved electronic copies of this document are considered uncontrolled			
 Healthy kids, healthy communities CompassionExcellenceCollaborationAccountabilityEquityRespect Neonatology Community Health Mental Health Perth Children's Hospital			

Appendix 1: De-escalation flowchart

